

AI BRAIN TUMOR DETECTION CENTER

Report ID: 925d3da3-9cbc-4c92-afa8-2e74f0233d3a
Date: 2026-06-15 14:46:00.530078



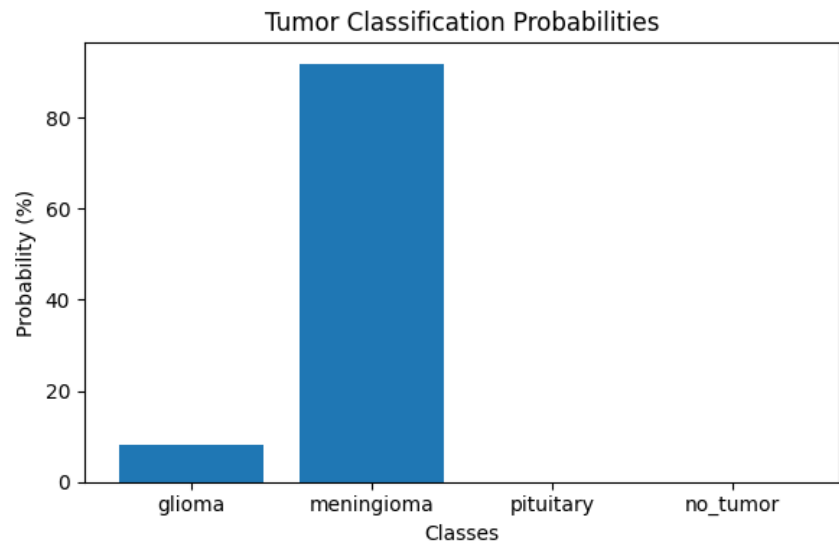
Patient Name	manudeep
Age	20
Email	manubattind
Tumor Type	meningioma
Confidence	91.86%

AI SUMMARY

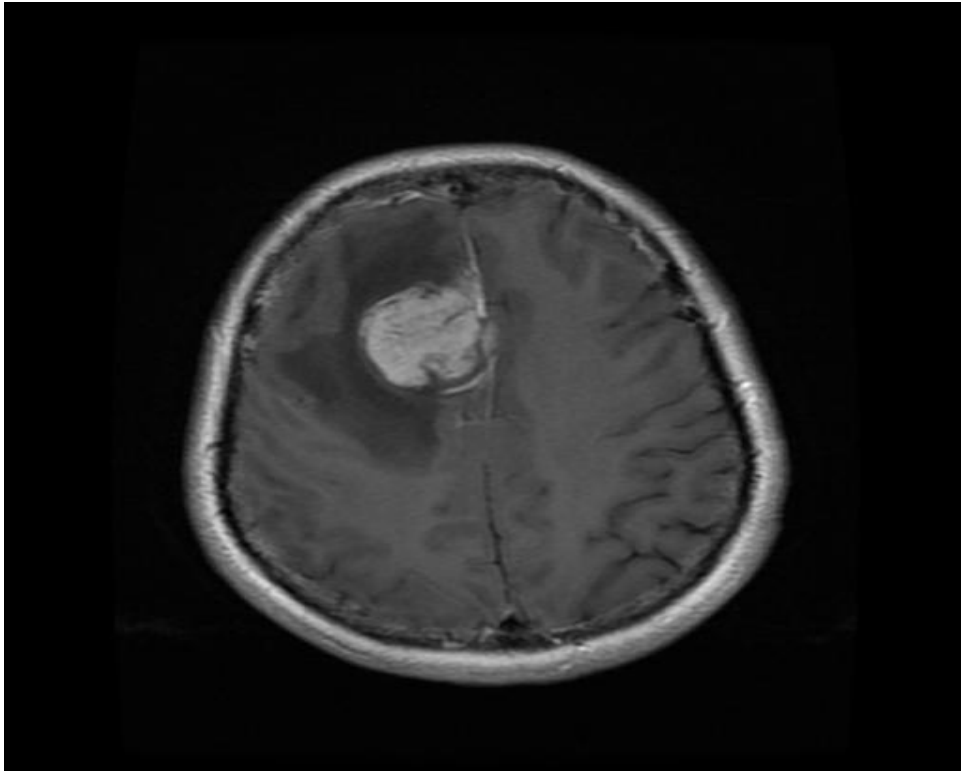
Brain MRI Classification Result: Tumor Type: meningioma Confidence: 91.86% **1. Tumor Explanation:** The AI analysis suggests a meningioma, a common type of primary brain tumor. Meningiomas originate from the meninges, the membranes surrounding the brain and spinal cord. They are typically slow-growing and most often benign (non-cancerous), though atypical or malignant forms can occur. **2. Possible Symptoms:** Symptoms depend on the tumor's size and location. Common manifestations may include headaches, seizures, vision changes, hearing loss, or weakness/numbness on one side of the body. Many meningiomas are asymptomatic and discovered incidentally. **3. Clinical Recommendations:** Given this finding, consultation with a neurosurgeon is highly recommended for expert evaluation. This may involve further imaging (e.g., higher-resolution MRI with contrast, CT), a comprehensive neurological examination, and discussion of management options, which range from observation to surgical resection or radiation therapy, tailored to the individual case. **4. Patient-Friendly Summary:** Your MRI shows what looks like a meningioma, a common, usually slow-growing growth on the brain's outer covering. While often not cancerous, it can cause symptoms like headaches or seizures depending on its location and size. The next important step is to see a brain specialist (neurosurgeon) to discuss what this means for you and the best way forward. This finding requires medical attention for proper evaluation, but is not a final diagnosis.

Probability Distribution

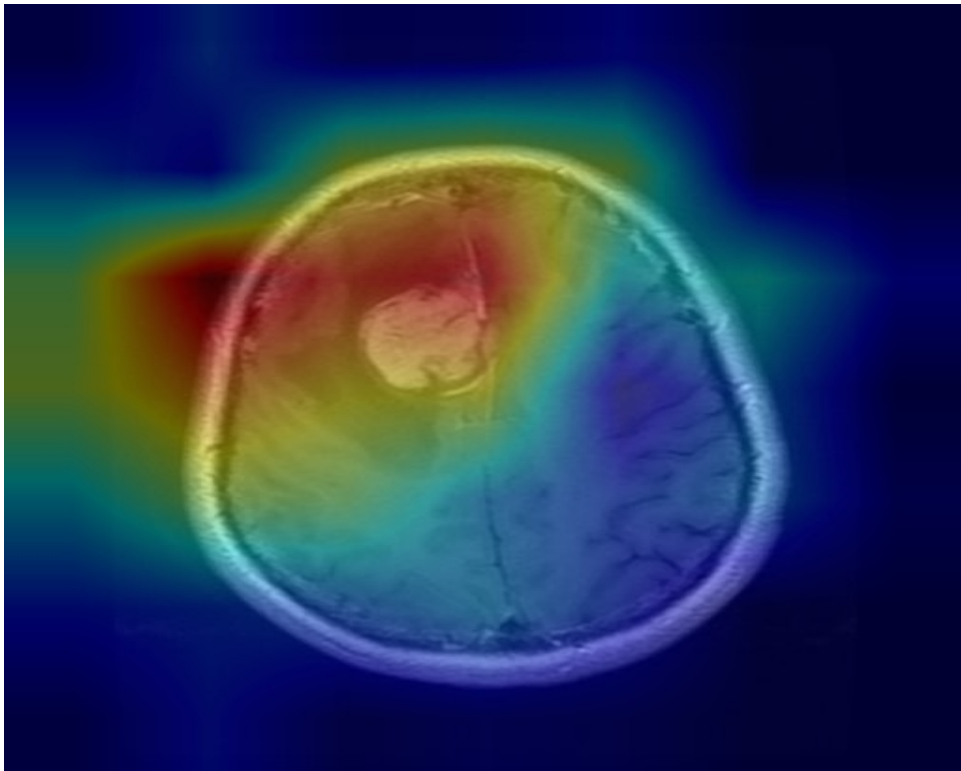
Class	Probability (%)
glioma	8.13
meningioma	91.86
pituitary	0.0
no_tumor	0.0



Original MRI Scan



Grad-CAM Visualization



Clinical Recommendation

Based on the AI analysis, the predicted class is **meningioma**. Further consultation with a neurologist/radiologist is recommended. Clinical correlation and additional investigations may be required.

Disclaimer:

This report is generated by an Artificial Intelligence system and is intended only for decision support. Final diagnosis should always be confirmed by a qualified healthcare professional.

Generated by AI Brain Tumor Detection Center